

PUBLIC HEALTH LEADERSHIP

ENUMERATION AND CHARACTERIZATION OF DEGREE AND CERTIFICATE PROGRAMS IN SCHOOLS AND PROGRAMS OF PUBLIC HEALTH

2025

CENTER FOR PUBLIC HEALTH WORKFORCE DEVELOPMENT
ASPPH

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AUTHORSHIP

Emily M. Burke, EdD, MPH, CPH
Senior Director of Workforce Development and Applied Practice
ASPPH

Jordan A. Fox, MPH
Manager of Public Health Workforce Initiatives
ASPPH

Brett Turner, MA
Doctor of Public Health Candidate
Harvard TH Chan School of Public Health

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FOR MORE INFORMATION:

ASPPH Center for Public Health Workforce Development

1615 L St. NW, Ste. 510
Washington, DC 20036
(202) 296-1099

workforce@aspph.org

EXECUTIVE SUMMARY

In 2022, the Association of Schools and Programs of Public Health (ASPPH) received funding from the U.S. Centers for Disease Control and Prevention (CDC) to engage academic public health and our practice partners to establish a national leadership training agenda for governmental public health. Bringing together an expert panel of representative perspectives, ASPPH developed the Public Health Leadership Competency Mapping and Training Agenda that establishes a framework for the application of training to increase the leadership competencies of the current and future public health workforce.

As a next step in exploring national public health leadership training opportunities, ASPPH analyzed data from the ASPPH Academic Program Finder and SOPHAS, the centralized application service for academic public health, identifying 44 public health leadership certificate and degree programs in ASPPH-member schools and programs of public health. These programs received over 681 applications for the 2022-2023 academic year. This report presents an analysis of these data as well as the results of key informant interviews with representatives from eight schools and programs of public health that offer public health leadership degree and/or certificate programs.

In the 2022-2023 SOPHAS application cycle, there were 681 applications for public health leadership degree and certificate programs, from 593 applicants. This represents 1% of total application volume for that SOPHAS application cycle. In that same application cycle, there were 69,155 applications to all other SOPHAS programs, from 26,261 applicants. Overall, applicants to public health leadership degree and and certificate programs in this cycle were older and more diverse compared with applicants that applied to other public health degree and certificate programs. To contextualize these data, eight ASPPH-member institutions were interviewed to further understand the structure, effectiveness, and applicant dynamics of education and training within public health leadership degree and certificate programs.

The findings of this report conclude that public health leadership programs are diverse in structure and delivery, and that establishing successful public health leadership programs begins with effective program design and thoughtful student recruitment. Despite facing infrastructure and financial challenges, these programs demonstrate a commitment to equipping students with the skills needed to address contemporary public health issues. Collaboration and sharing of best practices may further enhance leadership education and training opportunities and better prepare graduates for the evolving demands of the field of public health.



BACKGROUND & OVERVIEW

Public health practice continues to grow more complex due to a myriad of factors, including increased population density and diversity, as well as greater global connectedness (Hertelendy et al., 2022). The mounting threats to our global well-being have prompted the call for more robust leadership education and training within the discipline (Koh & Jacobson, 2009; Koh, 2009), and create a need for significant changes to public health pedagogy (Hertelendy et al., 2022). The specific challenges faced in the public health sector create a need for professionals trained in leadership to confidently guide the public through uncertain times. Public health leadership is a priority discipline that practicioners must learn in order to address demanding public health issues (Uno & Zakariasen, 2010).

Effective leaders are crucial in solving public health challenges and achieving progress in the field overall (Czabanowska & Babich, 2015). Public health professionals must be comfortable, no matter their position or level of seniority, taking on leadership responsibilities. While the theme of "leadership" is sweeping, incorporating specific skills and competencies into public health curricula will prepare students and practitioners to succeed in future public health leadership roles.

The American Public Health Association (APHA) states that public health leadership markedly functions to "convene people to solve complex problems" in these domains: systems-based views, collaborative/team based organizational cultures, diversity, community centrism and cultural humility, transdisciplinary approaches, and social movement building (APHA, 2022). The Public Health Foundation (PHF)'s Council on Linkages Between Academia and Public Health Practice (COL) developed the Core Competencies for Public Health Professionals, which was first adopted in 2001. COL pledged to revisit the competencies every three years to make necessary updates and revisions (COL, 2021). The 2010 updated version of the Core Competencies organizes competencies into eight domains, reflecting skill areas within public health:

Data Analytics and Assessment Skills	Policy Development and Program Planning Skills
Communication Skills	Health Equity Skills
Community Partnership Skills	Public Health Sciences Skills
Management and Finance Skills	Leadership and Systems Thinking Skills

In 2015, Grimm et al. conducted a qualitative analysis to determine domains and skills necessary for effective leadership in the field of public health. From their analysis, the researchers developed the following public health leadership domains and skills: community/organizational responsiveness, ability to inspire, results focused, social intellect, authenticity, and composure/balance (Grimm et al., 2015). These diverse yet interconnected competency domains illustrate the wide reach that leadership skills have in the field of public health.

CULTIVATING EFFECTIVE LEADERSHIP FOR PUBLIC HEALTH

Public health faces complex challenges that require skilled leadership, and practitioners must be equipped with leadership skills to be effective in the complicated public health environment (Lachance & Oxendine, 2015). However, few graduate degree or certificate programs are designed and delivered explicitly as public health leadership education and training programs, despite the call from practitioners for competency-based leadership training.

Post-secondary education is a popular option for public health education and training; however, an insufficient number of professionals in the field have obtained formal public health training through graduate degree programs (Uno & Zakariasen, 2010). There are no universal credential requirements for public health professionals, and many enter their public health careers having no training or background in health or public health, emphasizing the interdisciplinary nature of the profession (Uno & Zakariasen, 2010).

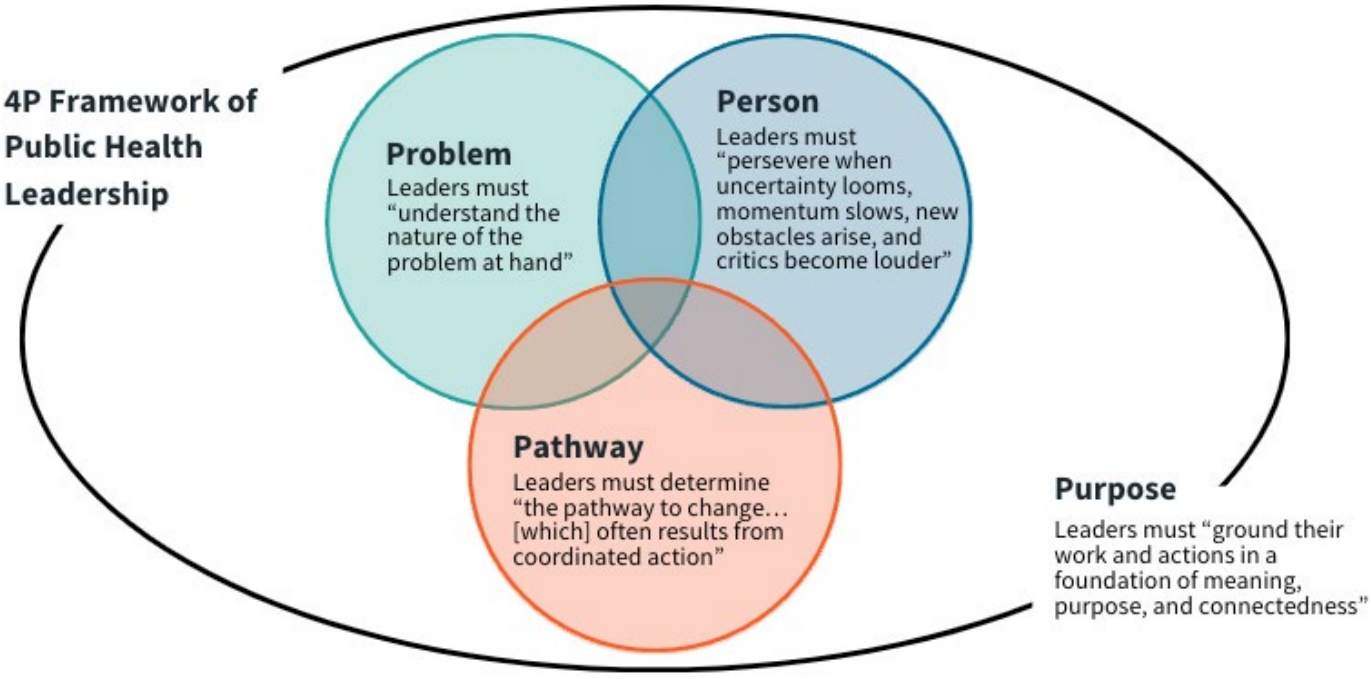
A 2022 study by Hertelendy et al. examined 179 Council on Education for Public Health (CEPH)-accredited Master of Public Health (MPH) programs (58 schools of public health and 121 programs of public health) to determine which MPH degree programs offered courses on the topics of leadership, crisis leadership, and crisis management during the 2021 academic year (Hertelendy et al., 2022). The researchers found that leadership courses were offered in 30 of the 58 schools of public health (67.2%) and 60 of the 121 programs of public health (49.6%) (Hertelendy et al., 2022).

The forthcoming analysis found even smaller numbers of degree and certificate programs designed for public health leadership education and training. Of the 5,158 public health degree and certificate programs observed, only 44 were public health leadership programs. Public health curricula must be revised to embed leadership competencies that prepare graduates to lead in complex, critical situations. Schools and programs of public health should provide opportunities for students to learn on the basis of commitment to lifelong learning, flexibility in design, practical public health experience, and recognition of the diverse needs of the population (Uno & Zakariasen, 2010).

The 2021 administration of the Public Health Workforce Interests and Needs Survey (PHWINS) assessed training needs among the United States governmental public health workforce across 10 strategic skill domains. The survey results revealed training needs expressed by supervisors, managers, and executives within the workforce. The 10 skill domains examined in the PHWINS survey are closely related to the key leadership competency themes identified by ASPPH, and emphasize the priority of developing these key leadership competencies among public health students in preparation for leadership roles in the workforce.

Some will argue that leaders are not created in the classroom, and that one becomes a leader via their efforts and experiences outside of academia. Nonetheless, academia is essential in assisting with this process and encouraging students to be lifelong-learners, an important characteristic of strong leaders (Uno & Zakariasen, 2010). However, leadership should not be considered one competency among many. Rather, it should be considered a framework for multiple competencies. Public health leadership education can and should stand alone as a specialized training and degree program.

Working with the ASPPH Public Health Leadership Expert Panel, the authors referenced published public health competencies, strategic frameworks, accreditation requirements, and job tasks and domains to identify and map leadership competencies and skills needed to address the complex public health landscape. For each identified leadership competency, ASPPH analyzed aspects of each identified leadership competency, including competency descriptions and key skills. The leadership competencies resulting from this analysis were then reviewed for key themes; ten key themes were identified and mapped into the categories of Problem, Person, Pathway, or Purpose, all existing within the ecosystem of Purpose in the 4P Framework of Public Health Leadership. The 4P Framework for Public Health Leadership requires an iterative process, wherein public health leaders continuously revisit their assumptions and strategies in response to evolving circumstances and requires flexibility, reflection, and adaptation in navigating the complexities of addressing public health challenges.



The competency mapping informed the development of a leadership training agenda for the governmental public health workforce, addressing emerging needs and trends related to public health leadership training and curricula. In mapping the competencies, the ASPPH Public Health Leadership Expert Panel identified areas for training opportunities related to the key themes and competencies, calling for the identification of gaps in available trainings and the development of new training opportunities to address such needs.

To better understand the current demand, availability, and curricula in public health leadership education and training, ASPPH analyzed the ASPPH Academic Program Finder, interviews with public health leadership program faculty and staff, and student application data. This analysis sought to answer the following three questions:

1. How many public health leadership-focused degree and certificate programs are currently available, and how do they differ in availability and structure from other programs at schools and programs of public health?

2. How are existing public health leadership-focused degree and certificate programs designed, and how do they train future public health leaders?

3. What is the volume of student demand for public health leadership-focused degree and certificate program training, and how do leadership program applicants differ from applicants to other programs?





METHODS

Phases

PHASE 1

Assessment of the landscape of public health leadership degree and certificate programs

PHASE 2

Analysis of student applications to public health leadership degree and certificate programs

PHASE 3

Interviews with public health leadership academic program representatives to assess practices and challenges in public health leadership degree and certificate programs

Landscape of Public Health Leadership Degree and Certificate Programs

Academic public health programs with a focus on public health leadership were identified using ASPPH's Academic Program Finder, and data were examined from the 2022-2023 academic year. The Academic Program Finder contained information for 5,158 distinct public health degree and certificate programs.

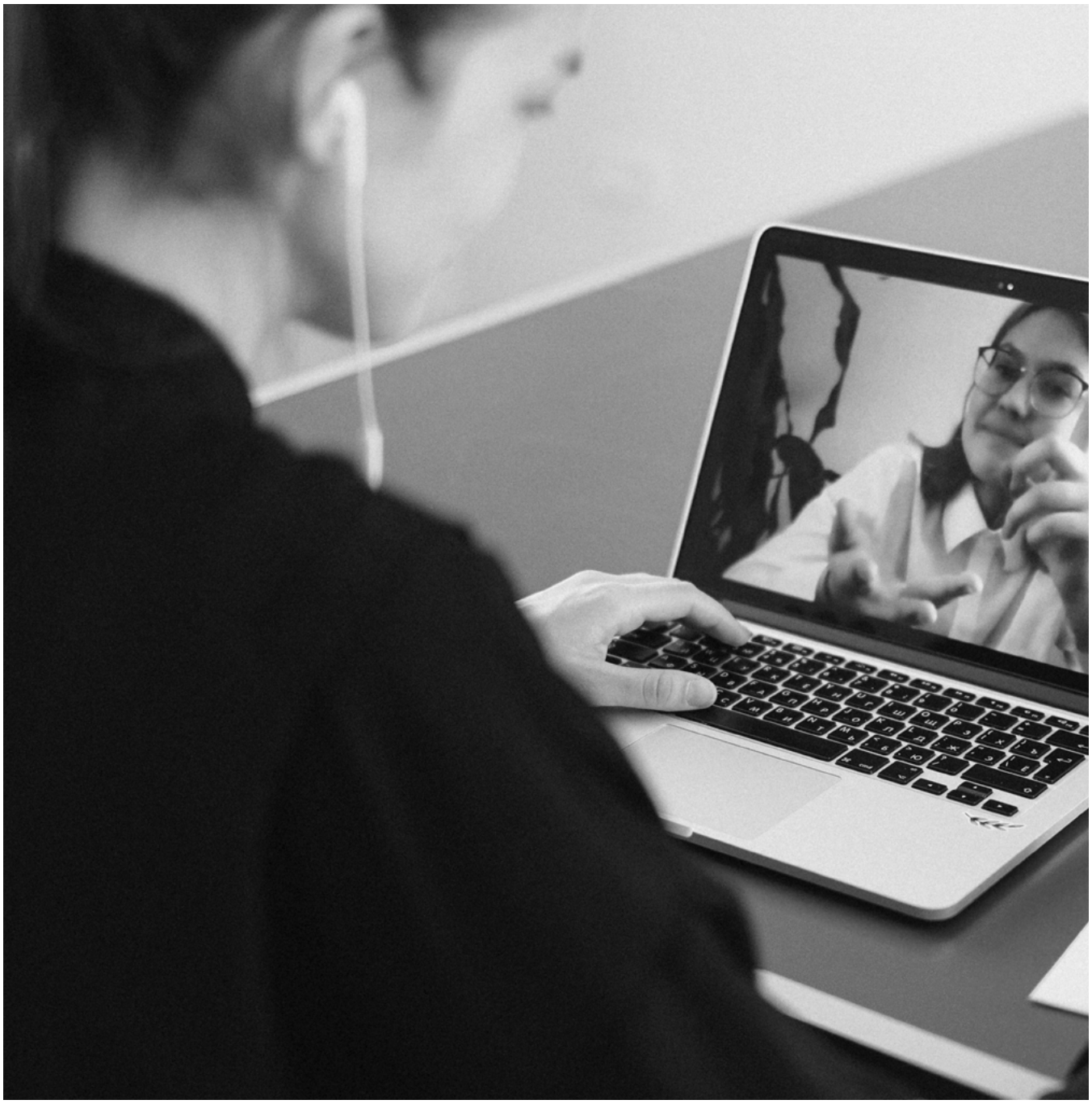
Programs listing "public health leadership" as their area of study were categorized as "leadership programs," while all other programs were grouped as "non-leadership programs." A comparative analysis was performed to assess differences between leadership and non-leadership public health certificate and degree programs, including number of programs offered at the academic institution, degree type distribution, GRE requirement rate, mode of delivery distribution, and geographic availability.

Summary of Applications to Public Health Leadership Degree and Certificate Programs

Data from the ASPPH Academic Program Finder were merged with SOPHAS application data for the 2022-2023 academic year. SOPHAS is the centralized application system for ASPPH-member schools and programs of public health. The merged data were analyzed to identify differences between applicant pools for leadership and non-leadership programs. The dataset included 69,155 applications to public health degree and certificate programs. A comparative analysis was conducted to identify demographic and characteristic differences between applicants to leadership versus non-leadership degree and certificate programs.

Practices and Challenges in Public Health Leadership Degree and Certificate Programs
Following the identification of public health leadership degree and certificate programs, representatives were contacted via email to participate in semi-structured interviews focusing on the leadership training aspects of their respective programs. Representatives were asked to share the interview invitation with other relevant faculty or staff if applicable.

Interviews were transcribed using the AI transcription service Otter.ai (Ottar.ai, 2024). Transcripts were then reviewed to summarize individual responses and identify key themes.





RESULTS

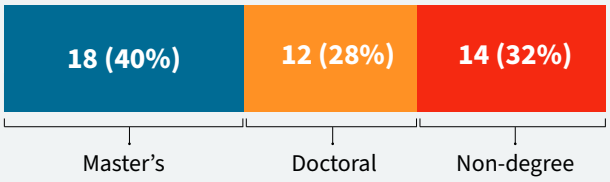
Of the 5,158 programs observed in the ASPPH Academic Program Finder for this period, 44 were identified as "Public Health Leadership". The five most common areas of study for non-leadership programs were General Public Health (n = 726), Health Policy and Management (n = 679), Epidemiology (n = 647), Behavioral Sciences (n = 627), and Environmental Science (n = 517). This does not suggest that non-leadership degree and certificate programs do not offer leadership training; however, an analysis of leadership curricula in non-leadership programs is beyond the scope of this analysis.

Doctoral and non-degree offerings are more prominent among public health leadership programs than among non-leadership programs. 40% of public health leadership programs are master's degree programs, 28% are doctoral programs, and 32% are non-degree programs. In comparison, 70% of non-leadership programs offer master's degrees, 12% offer doctoral degrees, and 18% offer non-degree training.

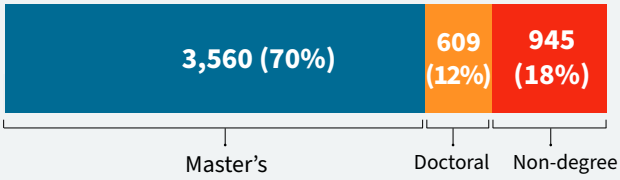
Public health leadership degree and certificate programs are more likely to be offered in online (36%) or hybrid (38%) formats, compared to non-leadership programs (18% online, 14% hybrid). Non-leadership programs are more likely to be offered in-person (68%) than leadership programs (24%).

DEGREE LEVEL

Public Health Leadership programs:

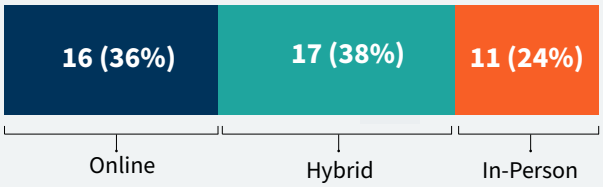


Non-leadership programs:

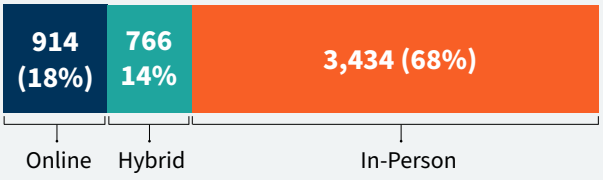


DEGREE DELIVERY

Public Health Leadership programs:



Non-leadership programs:



There were 681 applications to public health leadership degree and certificate programs from 593 applicants in academic year 2022-2023. This represents 1% of the total application volume for this SOPHAS application cycle. In contrast, there were 69,155 applications to all other SOPHAS programs, from 26,261 applicants.

APPLICANT DEMOGRAPHICS

This table displays the count of applicants to the identified public health leadership and certificate programs for the 2022-2023 SOPHAS application cycle.

	Leadership Programs - Doctoral (n = 403)	Leadership Programs - Master's (n = 191)
Age		
Mean Age	36 years	29 years
17-21	1 (0%)	21 (11%)
22-25	20 (5%)	58 (30%)
26-30	79 (20%)	42 (22%)
31-40	188 (47%)	41 (21%)
41+	115 (29%)	29 (15%)
Gender		
Female	276 (68%)	157 (82%)
Male	127 (32%)	34 (18%)
Race/Ethnicity of U.S. Citizens		
American Indian/Alaska Native	0 (0%)	2 (1%)
Asian	41 (16%)	31 (19%)
Black or African American	85 (32%)	15 (9%)
Hispanic/Latino	29 (11%)	19 (11%)
Native Hawaiian/Pacific Islander	0 (0%)	0 (0%)
White	90 (34%)	89 (53%)
Two or More Races	7 (3%)	7 (4%)
Unknown	11 (4%)	4 (2%)
Citizenship		
U.S. Citizens	263 (65%)	167 (87%)
International Citizens	140 (35%)	24 (13%)

ACCEPTANCE RATES

This table displays the count of application packages and accepted applications to the identified public health leadership and certificate programs for the 2022-2023 SOPHAS application cycle. Applicants may have submitted applications to more than one public health leadership degree or certificate program.

	Leadership Programs - Doctoral		Leadership Programs - Master's	
	Count of Applications	Count Accepted (% Accepted)	Count of Applications	Count Accepted (% Accepted)
Total	487	83 (17%)	194	143 (74%)
Age				
17-21	1	0 (0%)	21	16 (76%)
22-25	24	1 (4%)	59	50 (85%)
26-30	92	20 (22%)	43	31 (72%)
31-40	236	39 (17%)	42	29 (69%)
41+	134	23 (17%)	29	17 (59%)
Gender				
Female	328	62 (19%)	159	119 (75%)
Male	159	21 (13%)	35	24 (69%)
Race/Ethnicity of U.S. Citizens				
American Indian/Alaska Native	0	0 (0%)	2	0 (0%)
Asian	50	7 (14%)	31	20 (65%)
Black or African American	101	24 (24%)	16	10 (63%)
Hispanic/Latino	31	3 (10%)	20	14 (70%)
Native Hawaiian/ Pacific Islander	0	0 (0%)	0	0 (0%0
Two or More Races	8	1 (13%)	7	6 (86%)
White	110	30 (27%)	89	72 (81%)
Unknown	13	4 (31%)	4	4 (100%)
Citizenship				
U.S. Citizens	313	69 (22%)	169	126 (75%)
International Citizens	174	14 (8%)	25	17 (68%)

SEMI-STRUCTURED INTERVIEW PARTICIPANTS

University at Albany College of Integrated Health Sciences	Colorado School of Public Health	Georgia Southern University Jiann-Ping Hsu College of Public Health	Harvard T.H. Chan School of Public Health
Ohio State University College of Public Health	University of Arkansas for Medical Sciences Fay W. Boozman College of Public Health	University of North Carolina Gillings School of Global Public Health	University of South Florida College of Public Health

SEMI-STRUCTURED INTERVIEW THEMES

During 60-minute semi-structured interviews in May and June of 2024, one or more representative(s) from these schools of public health shared details about the objectives, learning experiences, structure, and other characteristics of their public health leadership degree and certificate programs.

- **LEADERSHIP CURRICULA**
- **PRACTICUM AND INTEGRATED LEARNING EXPERIENCES**
- **SELF-ASSESSMENT AND LEADERSHIP DEVELOPMENT**
- **COACHING AND MENTORSHIP**
- **PROGRAM DELIVERY**
- **STUDENT SELECTION**
- **PROGRAM INFRASTRUCTURE**

LEADERSHIP CURRICULA

Through a combination of required and elective courses, practicums, internships, integrative learning experiences, and more, programs train current and future leaders in the foundational skills of public health and in the competencies that are needed to serve as effective leaders.

The ASPPH member schools that participated in this study require courses that cover a wide range of topics, including foundational public health concepts (e.g. epidemiology, biostatistics, research methods), and concepts in management and strategy (e.g. communication, negotiation, organizational management, financial management). Variations amongst the programs include some placing greater emphasis on foundational public health science skills, with others requiring more coursework in management and strategy.

Core public health disciplines, such as epidemiology, health policy and management, and behavioral science, provide important foundational skills. However, segmenting disciplines in this manner may not reflect how skills are applied in practice, particularly for leaders who must apply knowledge and training from multiple disciplines simultaneously. Practice-based learning provides opportunities for the integration of skills from multiple disciplines at once.

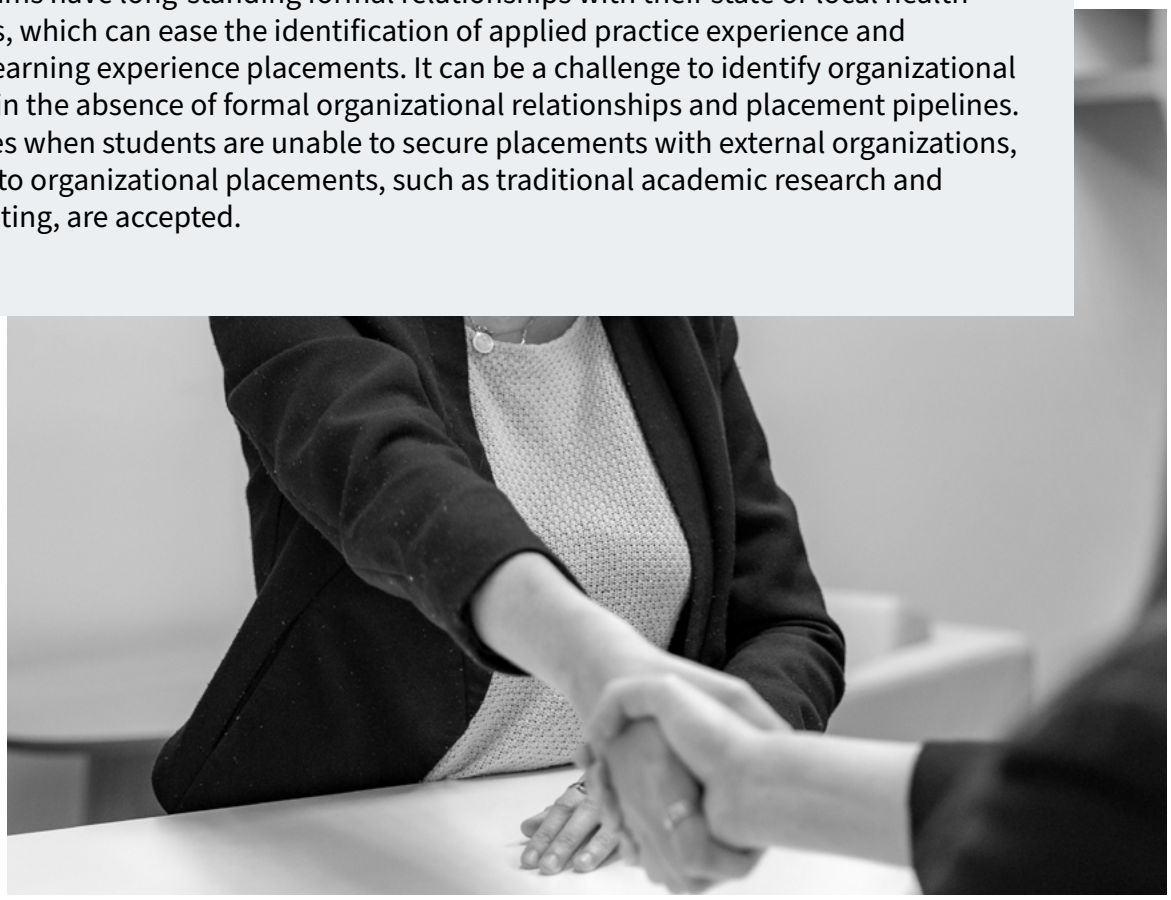


PRACTICUM AND INTEGRATED LEARNING EXPERIENCES

Applied practice and integrative learning experiences allow students to apply the skills and competencies learned in the classroom to practical situations. By their nature, these practical experiences overcome some of the previously mentioned limitations of discipline-specific coursework, as students are immersed in organizational settings where they can apply their learnings from across academic disciplines into real-world scenarios.

Individual applied practice and integrative learning experiences vary in structure, and may involve aspects such as project-based work, community-engaged research, and program evaluation. These experiences are intended to, at an advanced level, give students the opportunity to "collaborate with practitioners, develop leadership competencies, and contribute to the field" (Council on Education for Public Health, 2021). Strong partnerships with organizations that serve as potential practice sites are important for identifying meaningful placement opportunities. Some students who enter public health leadership degree and certificate programs as working professionals establish applied practice experiences and integrative learning experiences with a current or past employer, while others secure placements with new-to-them organizations. Faculty members with experience in public health practice closely advise student learning and ensure high-quality engagement with partner organizations.

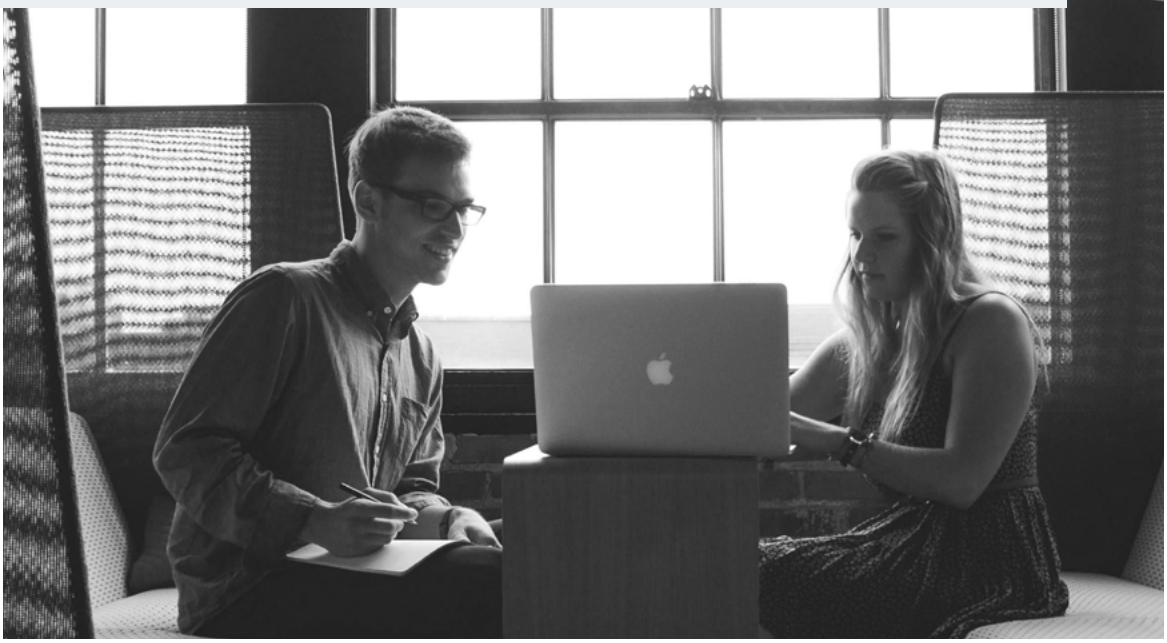
Some programs have long-standing formal relationships with their state or local health departments, which can ease the identification of applied practice experience and integrative learning experience placements. It can be a challenge to identify organizational placements in the absence of formal organizational relationships and placement pipelines. In some cases when students are unable to secure placements with external organizations, alternatives to organizational placements, such as traditional academic research and scientific writing, are accepted.



SELF-ASSESSMENT AND LEADERSHIP DEVELOPMENT

Many public health leadership degree and certificate programs incorporate self-assessment tools and exercises to help students better understand their strengths, weaknesses, values, and behavioral patterns. Assessment tools mentioned by participants include the DISC assessment, Myers-Briggs, EQI, Clifton StrengthsFinder, and other leadership style inventories. These assessments are often used in leadership courses to facilitate reflection on personal leadership styles and preferences. Several influential leadership frameworks point to self-awareness as a foundational competency for developing strong interpersonal skills (Goleman, 1998), authentic values-driven leadership (George et al., 2007), and the ability to navigate complexity and adaptive challenge (Heifetz & Laurie, 2001).

The Harvard T.H. Chan School of Public Health incorporates 360-degree feedback and peer coaching to provide students with additional perspectives on their leadership. Reflective writing assignments also prompt self-awareness. At the University of North Carolina Gillings School of Global Public Health, students complete personal and social identity wheels to examine how their social identities influence their leadership approach. The University at Albany College of Integrated Health Sciences has students review competencies and create personalized development plans based on the competencies they want to focus on developing further. This process allows students to direct their growth in personally relevant ways that are explicitly connected to leadership competencies.

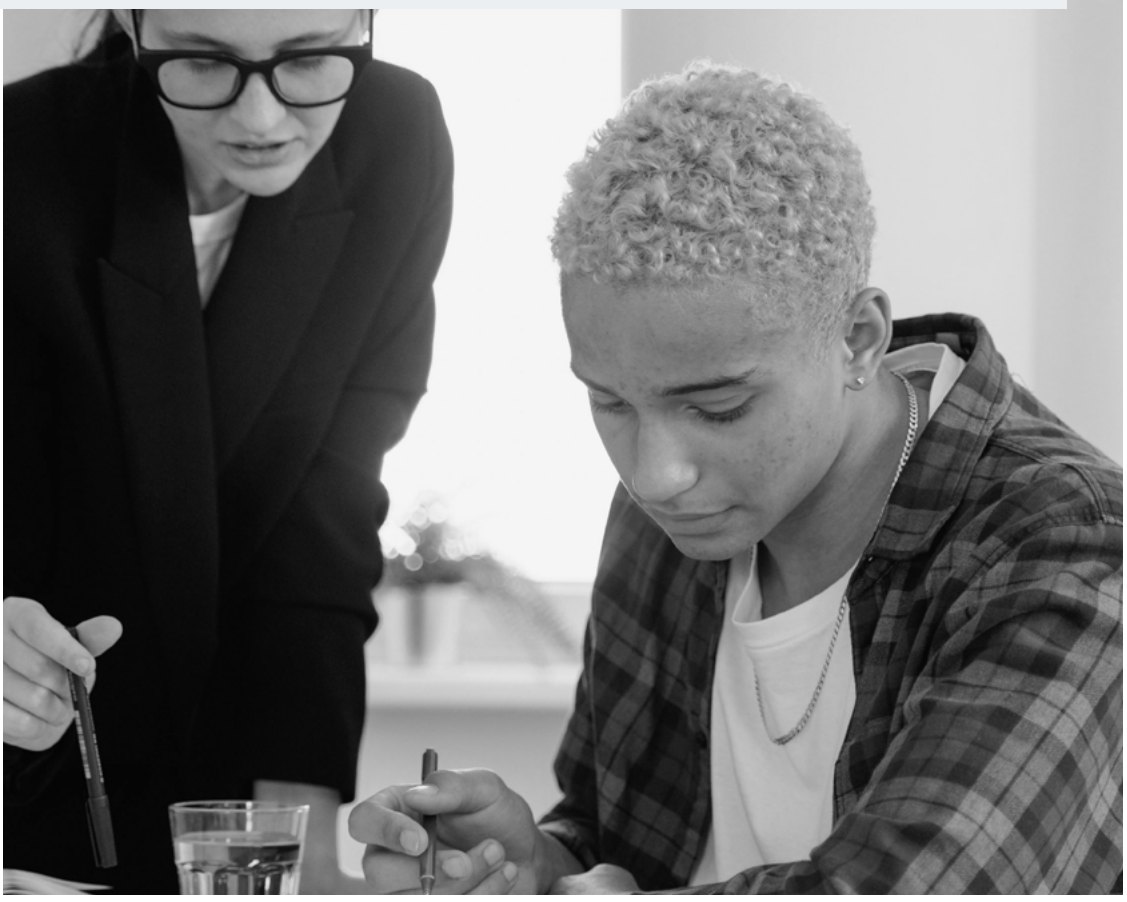


COACHING & MENTORSHIP

Coaches and mentors support students' leadership development beyond the classroom and serve as trusted advisors for personal and professional growth. Coaching and mentorship take various forms across the public health leadership degree and certificate programs, including external coaching professionals, faculty-based academic advisors, field-based mentors and preceptors, and peer-group coaching.

Coaching provides students with one-on-one time to reflect on their leadership strengths and opportunities for growth. Coaches provide feedback and guidance to help students develop action plans. Coaches also help students process and navigate their applied practice or integrated learning experiences, and can serve as mentors in areas where they have technical expertise. Some programs include peer coaching, through which students meet in pairs or small groups to learn from each other and to practice providing and receiving feedback.

Many programs ask applicants during the interview stage to consider their long-term goals and how completing the public health leadership degree or certificate program will contribute to those goals. Applicant responses are considered in candidate selection and serve as a starting point for ongoing coaching after enrollment in the degree or certificate program.



PROGRAM DELIVERY

Whether programs operate under an online, in-person, or hybrid model, substantial effort is required to facilitate high-quality learning experiences. One format may be preferred to another based on the goals of the program, and the relative benefits and challenges each mode of delivery presents in attaining those goals.

ONLINE PROGRAMS

Online programs help to maximize accessibility for working professionals and enable students to learn while continuing their professional and/or personal responsibilities. Online programs may be able to operate at reduced costs by eliminating infrastructure expenses such as classroom space, and may help reduce the cost of attendance for students by eliminating housing, travel, or other expenses.

Representatives from online programs reported difficulty developing a strong sense of community and connection among students. In addition, schedule coordination between students may present logistical challenges with group work and team-based projects. Online programs may also present technology barriers for students with less technical expertise or restricted access to technology, and may make it difficult for instructors to assess student progress, build rapport, and provide students with real-time feedback.

IN-PERSON PROGRAMS

The immersive learning environment of in-person programs is conducive to community building, creating more opportunities for student connection, collaboration, and impromptu discussion. Instructors may be able to more easily recognize the level of student understanding of material and provide real-time feedback and adjustment.

In-person programs, however, are generally less accessible for working professionals, those with familial obligations, and students not located near the campus. Higher costs associated with travel, living expenses, and lost wages also compromise access to these programs, potentially resulting in smaller and less diverse applicant pools.

Some in-person programs allow students to take online courses to fulfill their credit requirements. In-person programs may promote supplemental online experiences, such as webinars related to professional development goals. However, programs are typically either in-person or online in nature, and none of the programs interviewed had an explicitly hybrid model of operation.

STUDENT SELECTION

Public health leadership degree and certificate programs weigh various factors during student selection. The weight of each factor differs from program to program and can vary based on the mode of delivery. Online programs, for example, may prioritize work experience more heavily if they are targeting executives or mid-career professionals as their student audience. In-person programs, on the other hand, may have more flexibility in candidate profiles or may assess the feasibility of full-time immersion in an in-person program.

Below are the primary factors considered by public health leadership degree and certificate programs during candidate selection:

Academic background is an important indicator of a student's ability to succeed in the academic aspects of the program. Program faculty and staff assess past performance in quantitative courses, such as statistics, as evidence of the candidate's ability to successfully complete quantitative coursework of the public health leadership degree or certificate program. However, academic factors are not reviewed in isolation, and programs may admit students who are below GPA cutoffs if other factors are strong – especially if academic history is older and therefore less indicative of recent performance, or if there is substantial professional experience indicating proficiency in relevant skills.

Work experience is often one of the most heavily weighted factors for evaluating candidates for leadership programs. Most programs require three to five years of professional experience, preferably in a leadership role. Some programs hold this as a strict requirement, while others make exceptions for exceptional candidates, acknowledging the benefit of having student cohorts with diversity in age and experience.

Personal statements are assessed for clear statements of purpose and clarity in how the degree will fit into the candidate's career goals. An explicit interest in leadership and in public health are expected. For example, a state health department-funded public health leadership degree program that serves as a workforce pipeline program uses personal statements to identify interest in a career within the health department.

Recommendation letters are considered by all programs, and provide insight into candidate qualifications and overall fit for the program.

GRE requirements have been eliminated by most public health leadership degree and certificate programs; however, applicants are able to submit GRE scores voluntarily. In these cases, scores are not assessed in a punitive way but are instead used as evidence of academic ability – especially for quantitative skills – when GPA does not meet the stated GPA cutoff.

PROGRAM INFRASTRUCTURE

Semi-structured interview participants indicated some key infrastructure needs and challenges related to operating public health leadership degree and certificate programs. Financial constraints, faculty incentives to prioritize research, difficulty developing community and organizational partnerships, and technology needs were all cited as infrastructure challenges.

Faculty recruitment and retention can be difficult given the need for faculty with both practice and academic experience to teach and mentor public health leaders. Further, participants cited difficulty recruiting and retaining faculty to teach rather than conduct research.

Funding is limited for student tuition support and program operations. MPH and DrPH programs - being practice oriented – typically do not have the support of research dollars, which fund student tuition and operations for PhD students and research departments.

Organizational partnerships are important for practicum placements but can be a challenge to develop and maintain.

Technology infrastructure investments are needed to effectively support online programs through learning management systems, virtual classrooms, instructional designers, etc. Additionally, online programs may require additional staff time for individual student support to account for the lack of in-person or synchronous classroom support.

Interdisciplinary collaboration is needed to provide integrated learning experiences that teach student's skills in contexts that resemble real-world practice. Academic department-specific budgeting and faculty assignment can make cross-departmental collaboration challenging.

CONCLUSION

Establishing successful public health leadership degree and certificate programs begins with thoughtful program design and student recruitment. While the programs discussed in this report are diverse in structure and delivery, a commonality is each programs' dedication to equipping students with the skills needed to address contemporary public health issues, often despite financial and infrastructure challenges. Interviews with faculty from public health leadership degree and certificate programs provided valuable insights into the innovative ways that schools and programs of public health are training the next generation of public health leaders. Our national, institutional, and instructional recommendations for the creation and management of successful public health leadership programs were a result of these conversations.

NATIONAL LEVEL

National organizations and agencies should establish standardized competency frameworks, allocate funding and resources to public health leadership degree and certificate programs, and facilitate and promote the sharing of best practices and successful implementation models and curricula across academic institutions.

1. **Development of Standardized Leadership Competency Frameworks:** Establish clear guidelines and competencies for public health leadership that can be adopted across various workforce training and education programs to ensure consistency and quality in training.

2. **Funding and Support for Public Health Leadership Degree and Certificate Programs:** Allocate funding and resources to workforce training and education programs that emphasize public health leadership, recognizing the importance of these skills in the field.

3. **Promotion of Best Practices:** Create platforms for convening students and faculty from public health leadership degree and certificate programs, where they can share best practices and successful implementation models and curricula that can be replicated or adapted across schools and programs of public health.

During interviews, faculty from public health leadership degree and certificate programs stressed the importance of collaboration between academic programs and public health agencies and employers. Such partnerships ensure that curricula remain relevant and aligned with the ever-evolving demands of the field. Sharing best practices and fostering continuous dialogue between these stakeholders enhances the quality and impact of public health leadership training, preparing graduates to assume pivotal roles in the workforce.

INSTITUTIONAL LEVEL

Schools and programs of public health should develop public health leadership curricula for widespread adoption to ensure consistency and quality of training, prioritize funding and resources for public health leadership degrees and certificates, and actively contribute to the sharing of best practices and successful implementation models and curricula.

1. **Establish Leadership-Focused Degree Programs:** Design and deliver an interdisciplinary public health leadership curriculum that incorporates leadership competencies and prioritizes experiential learning to prepare students with the necessary skills to serve as effective leaders who can navigate complex challenges in public health practice.

2. **Integrate Leadership into Existing Programs:** Embed leadership training into existing public health degree and certificate programs by offering courses that focus on key competencies and leadership development, recognizing that students across disciplines will need leadership skills to be effective in the public health workforce.

3. **Support for Self-Assessment and Professional Development:** Encourage student self-assessment and professional development planning to cultivate and foster self-awareness, interpersonal effectiveness, and sense of purpose as an integral part of leadership development.

In interviews, faculty from public health leadership degree and certificate programs expressed a dedication to approaching education comprehensively at the institutional level. Students gain a foundation in public health disciplines while engaging in self-assessment and leadership development planning, which enables them to tailor learning experiences to their unique career aspirations. These elements are bolstered by structured support systems, including coaches, mentors, and peer networks, which are critical in fostering a collaborative and supportive learning environment.

INSTRUCTIONAL LEVEL

Public health leadership instructors should integrate interdisciplinary learning, employ active learning techniques such as simulations and case studies, and provide mentorship and real-world experiences through applied practice and integrative opportunities.

1. **Interdisciplinary Learning:** Create opportunities for students to learn public health leadership skills through interdisciplinary projects and partnerships which reflect the interdisciplinary nature of applied public health practice.
2. **Use of Active Learning Techniques:** Employ teaching strategies that foster active learning, such as simulations, role play, case studies, and project-based work, to deepen learning and enhance the development of public health leadership skills.
3. **Mentorship and Real-World Experiences:** Facilitate mentorship programs and provide opportunities for real-world leadership training through applied practice and integrative learning experiences, allowing students to apply their skills in public health practice settings.

The commitment of faculty members at the instructional level, as discussed during interview sessions, is another defining feature of successful leadership programs. Faculty not only impart knowledge but ideally model the skills and behaviors necessary for effective leadership. Such dedication ensures that graduates are equipped to navigate an evolving public health landscape, and address complex issues such as health equity, emergency preparedness, and community engagement. Moreover, applied practice experiences and integrative learning opportunities ensure that theoretical knowledge is successfully translated into practical solutions for real-world public health challenges.

Quality leadership training is imperative to advancing the skills of the public health workforce and ensuring that the field is equipped to tackle pressing health challenges as they arise. Academic programs offering public health leadership degrees and certificates play a pivotal role in cultivating the next generation of public health leaders. In a dynamic and challenging field like public health, robust leadership training is not just beneficial—it is essential. By investing in thoughtfully designed leadership programs, the public health sector ensures it has a pipeline of skilled, adaptable, and visionary leaders ready to address current and future challenges.



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